



RIVETING DEBATE

JULY 22: Niketa Mehta's 24-week foetus is diagnosed with a congenital heart block along with transposition of great vessels, which means that the child's aorta and pulmonary artery are malpositioned and the heart rate is less than normal. They are told that a foetus older than 20 weeks cannot be aborted.

JULY 24: Niketa and Haresh Mehta, along with gynaecologist Nikhil Datar, approach the Bombay High Court under the MTP Act (1971) seeking permission to abort the foetus.

JULY 29: Court orders setting up of medical committee. The first report was submitted on August 1.

AUGUST 4: The second and final report submitted. The high court rules against the Mehtas saying there is no need to terminate the pregnancy as the risks were not proven.

IN A QUANDARY: Niketa and Haresh Mehta outside the court

Niketa's reports are disconcerted because they believe that the issue plaguing them does not pertain to the baby's survival but its perfection and relative quality of life. They claim this is more to do with the social stigma and costs rather than concern over a possible deformity. After the court ruled against their petition, Niketa said, "I am aware that the baby will survive but I am concerned about the quality of life of the baby, which is why I went to court. Even though the judgement is not in my favour I do not regret going to the court."

Quashing the fears of Niketa's doctors, Subnis says: "Only 2 per cent of the congenital heart block cases require a pacemaker and the chances of the child suffering mentally or having a

physical deformity do not arise at this point. We have seen a large number of patients with this problem, but they all lead a normal life without ever having to undergo surgery." Subnis is supported by Dr Arun Mehra, cardiac surgeon at Mumbai's Breach Candy and Jaslok hospitals, who says that people with this condition live a normal life but may develop some problems after the age of 40. "Only some require a pacemaker that has to be changed every 5-6 years," he explains.

So for now, Niketa will have to live with the ruling and the baby, but the bold stance of the Mehtas and their lawyers have triggered social and medical debates in the country. Medical experts believe that this case has more to do with a societal problem and not medical because there are no indications that the child will be mentally un-

stable or handicapped. At a social level, the Mehtas seem to have garnered the sympathy of the society at large, which supports their decision to abort a baby with "deformities". But given the watered-down verdict handed by cardiologists, the crucial question that arises is whether parents should be given a licence to choose perfect babies and abort those with the slightest problem.

The Mehtas may have hoped that their petition would force the court to consider an abortion in special cases after the stipulated 20-week deadline, where the foetus is detected with severe deformities. The subject of abortion is hugely touchy even in the developed countries. In the US, abortion is currently legal but most states have passed laws restricting late-term abortions. India's MTP Act of allowing abortion before 20 weeks comes with its own complications. "The reason is that till 20 weeks, a foetus is not viable to survive outside of the womb, but after 20 weeks, though very rarely, it can live independently," says Dr Kiran Coelho, head of the gynaecology department, Lilavati Hospital, Mumbai. So in a case where labour can be induced, aborting after 20 weeks would amount to infanticide. In India, the Act was meant to encourage contraceptive use under the Family Planning Programme and was a back-stop for failure. However, with the Act being misused, as technology enabled people to find out the sex of the baby, the laws had to be tightened to deter people from abusing the MTP Act.

Considering that there is no provision under the law to abort if the unborn baby has deformities after 20 weeks, medical experts believe that while there is room for debate on this issue, there are dangers associated with relaxing the law, especially in a country which has the highest rate of female foeticide. "I get a high number of sex determination requests and increasing the abortion deadline might do more harm. If the law has to be changed at all, it has to be for very special cases where the child has an organ, like a bladder or kidney, missing and is sure to lead a vegetable existence," says Coelho. While the legislature mulls over this case, parents like the Mehtas will have to accept their fate and pin their faith in medical science. ■